

SECTION I

Trauma and IFS

CHAPTER 1

Complex PTSD and IFS

RELATIONAL TRAUMA

I want to even the playing field right out of the gate. I'm not a big fan of the "us versus them" mentality that says, "I'm the doctor and you're the patient," "I'm a healer and you're a victim," or "I'm the trauma expert and you have a trauma history." I believe that we all have been traumatized in some form at some point in our lives, that we each carry wounds deep within ourselves, and that everyone works hard to keep the pain from those experiences at bay. What's more, we're each susceptible to being triggered and to overreacting when our wounds resurface, and we all have what IFS refers to as the "Self"—our core, our essence, our internal compass that possesses inherent wisdom and healing capacity.

Complex PTSD, in its most basic form, is a relational violation that disrupts our sense of trust and safety in the world and limits access to our Self-energy. Healing relational trauma puts us back in touch with our natural core and permits us to love and connect again with others. We've all felt the difference between being alone and being lonely, or spending time by yourself contentedly versus feeling isolated and yearning for some form of connection. Those who are more introverted, for example, enjoy spending time alone. It rejuvenates them in a way that allows for a peaceful and quiet internal connection. This is different from the pain, isolation, and detachment that accompanies relational trauma and wounding (e.g., feeling the hurt and betrayal of rejection, a verbal critical attack, or sexual violation).

WHAT'S COMPLEX ABOUT COMPLEX PTSD?

There are several things that make working with someone who has complex PTSD so challenging. One of them is the approach to therapy itself. When someone suffers from repeated relational trauma, they struggle with closeness, trust, and safety in relationships. That same person seeks therapy with the unspoken intention of forming a close, intimate relationship with you: their therapist. We carry the hope that they will feel safe enough to share their deepest thoughts, feelings, and fears with us so we can help them resolve their experience of relational betrayal.

However, this is a premise that virtually guarantees failure. Why? Because it is unrealistic to expect that people who have been violated by relationships will expose their vulnerabilities to a total stranger without repercussions. Under these circumstances, traumatic reenactments are practically unavoidable.

Working with relational trauma inevitably triggers a client's wounds and activates our own relational history and personal wounding. It also rekindles our personal struggles with closeness and intimacy. Remember: You likely have caretaking parts that are rooted in protecting one of your relational wounds. You probably wouldn't have chosen this profession if you weren't on some level working through your own personal relational histories, right? Yet it feels safer to help someone else with their problems than to focus on our own, and we can feel good about ourselves to boot because we're helping others. But this isn't exactly a win-win. Even though being a therapist is a noble and satisfying profession, with relational trauma, each party's painful past is likely to get reactivated and complicate the intention of helping our client heal. It's certainly possible to achieve, but it's typically much more challenging than you'd expect.

This is one of the primary reasons I wanted to write this book: to help people better navigate this complicated journey of resolving relational trauma. Gratefully, I can relate the value of my experience—as a result of my own therapy and of working closely with my clients, I have been able to heal much of my own personal trauma.

Working with trauma can be difficult, especially when it activates our own histories. It's also problematic when severe symptoms or extreme reactions surface in the treatment. For example, many clients will feel suicidal at some point in their treatment. Some will resort to self-cutting in order to soothe themselves; substance abuse is also common, as are panic attacks, depression, dissociation, and shame. *Extreme experiences warrant extreme responses.*

Attachment trauma that developed in childhood often gets reactivated in therapy, triggering a desperate need for connection, a fear of closeness, and panic related to abandonment and loss. Boundary issues inevitably get stirred up with relational trauma as well. In my experience, a therapist can tend to fall into one of two places regarding boundary issues: too much or too little. When your parts are triggered around boundaries, you can overcompensate by *offering too much*, trying to rescue them by recommending things that are outside the therapeutic norm. Alternatively, you can become *rigid and controlling* in an attempt to protect yourself from the overwhelming neediness of your client's parts.

Trauma is rarely simple and straightforward as it relates to symptom production. Many survivors of trauma also struggle with depression, anxiety, panic, substance abuse, bipolar disorder, eating

issues, obsessive-compulsive disorder (OCD), and even psychosis in addition to their PTSD. Multiple comorbidities are common with clients who have been traumatized, and these biological abnormalities need to be managed to help our clients effectively address and fully heal the emotional effects of their abuse. All of these factors speak to the complicated nature of treating complex PTSD. And we as therapists need to constantly monitor our reactions, which are commonly activated throughout this process.

The following list summarizes some of the complications involved in treating complex PTSD.

- **The pitfalls of the therapy relationship**
- **The activation of therapist parts**
- **The presence of extreme symptoms and reactions in clients**
- **The reenactment of early attachment trauma**
- **The struggle with boundary-related issues**
- **The frequency of comorbid conditions**

SITTING WITH THE HORRIFIC

No matter what therapy model you practice, it's challenging to work with someone who has just described their past sexual, physical, verbal, emotional, or neglect trauma in horrific detail. Recollections like "At that moment, I thought I was going to die," "I was suffocating and gasping for air as he was on top of me," "The image of her lying facedown on the floor in a pool of blood will be embedded forever in my mind," and "As I heard each gunshot, I kept thinking to myself that the next one is for me" are jarring and disturbing to hear aloud. Listening to and processing these horrendous client memories is difficult, no matter how experienced you are in treating trauma. I will never forget what Bessel van der Kolk said during one of our team meetings at his Trauma Center many years ago: "Trauma survivors don't fear the worst-case scenario. They know it because they've lived the unthinkable." How right he is.

Let's face the facts: This is hard but rewarding work. I believe we're all drawn to this field for primarily personal reasons—myself included—and that sitting with someone as they describe and relive the unthinkable in our presence is tough to do and often activates our trauma. Yet, while I'm not perfect at this, over my career I've found that the more I heal my own trauma, the easier it is for me to work with my clients and help them process their horrific experiences. I'm better positioned to

stay present, maintain a compassionate stance, and avoid getting so empathically overwhelmed like I used to. IFS has helped me clear out so much of my traumatic history and exorcise my demons that I'm better able to treat clients without getting triggered myself or trying to rescue them as I did in the past. Make no mistake: It has taken me several years and a lot of personal therapy to arrive at this safer, calmer, and clearer place. But today, after unburdening many of my wounds and reconciling my negative experiences, I'm better able to serve my clients.

THE UNIQUE CONTRIBUTION OF IFS

The IFS model of therapy is rooted in systems thinking and the multiplicity of the mind. It believes that all individuals have what is called a *Self* (our core, soul, or internal leader), which doesn't need to be cultivated or developed; we are born with it. It also acknowledges that we have *parts* or aspects of our personality that we are born with too, which have positive intentions and are not a sign of pathology. With trauma, parts are forced to take on extreme protective roles or to carry painful experiences.

IFS has a unique way of addressing and dealing with trauma, and for me it's the most complete model I've come across to date. It deals with the *cognitive* by addressing and unloading distorted thoughts and beliefs, it incorporates the *body* as it supports the movement and discharge of physical sensations, and it facilitates the release of painful *feelings* (such as unworthiness, loneliness, and unlovability) by healing the wounds that parts of us carry. IFS helps release parts from their extreme roles and repairs the chasm created between parts and the Self as a result of the trauma. Parts are then able to restore trust in Self-leadership and fluidly integrate back into the whole system. IFS incorporates spirituality and promotes a humane, non-pathologizing, openhearted, and loving approach to healing the pain associated with repeated relational violations. I can personally attest to its power and efficacy. Being part of the solution, with love and compassion, perfectly aligns with who I am and who I want to be in and for the world.

GLOSSARY OF IFS TERMS*

5 P's: The qualities of a skilled IFS therapist: presence, patience, persistence, perspective, and playfulness.

6 F's: The steps used when working with protective parts: find the part, focus on it, flesh it out, feel toward it, befriend it, and find out its fear.

8 C's: The qualities of Self-energy: compassion, curiosity, calm, clarity, connectedness, confidence, courage, and creativity.

Blended: When a part takes over, making it indistinguishable from another part or from the Self.

Burdened: When a part holds negative beliefs (e.g., “I’m unlovable,” “I’m worthless”), intense trauma-related feelings (e.g., terror, panic, shame, rage), or physical sensations (e.g., body memories, visual flashbacks).

Direct access: A method of communication with parts. When a part is blended and the client’s Self is not accessible, the therapist speaks directly to the client’s part. This is an alternative form of communication when internal communication or *insight* is blocked.

Do-over: One of the steps of the unburdening process. After the Self has witnessed what an exiled part wants to share, the Self joins the part, going back to a time in the part’s past, and offers the part a fully corrective experience.

Exile (wounded part): A part that carries painful thoughts, feelings, and physical sensations as a result of traumatic or overwhelming experiences. The client expends a great deal of energy to keep exiled parts out of conscious awareness. Examples include parts that feel alone, unloved, shamed, or worthless.

Firefighter (reactive/extreme part): A protective part that typically emerges after a wounded part is triggered. Firefighters respond in an extreme and sometimes overreactive fashion, typically don’t care about the well-being of others and will do whatever is necessary to end an exiled part’s pain. Examples of firefighter responses include suicide, cutting, drinking, bingeing, raging, shaming, and dissociating.

Integration: One of the steps of the unburdening process. After the exiled part is healed, we bring protective parts back and suggest the release of their protective role.

Internal communication (insight): The primary approach therapists use to help clients communicate with their parts. The client's Self is able to listen to and communicate with a part. When parts block internal communication, we use direct access.

Invitation to exiled parts: One of the steps of the unburdening process. After the exiled part unloads the burden, it can internalize new qualities with which it chooses to fill the space the wound left behind.

Invitation to protective parts: An offer to a protective part when requesting permission to access a wounded part. During the invitation, the therapist can help guarantee that another part will not overwhelm the system, will propose a new role for the protective part, and will offer to heal the wound.

Manager (preventive part): One of two protective parts. The manager's goal is to keep exiled parts hidden by making sure the wound never gets activated. Manager types include conflict-avoidant, intellectual, obsessing, worrying, and hardworking parts.

Overwhelm: A primary fear in trauma and one of the offers to a protective part in the invitation. During the invitation, the therapist will directly ask an intense part not to overwhelm the system, which can make a protective part more willing to allow access to a wound that needs to be healed.

Parts: Separate mental states or subpersonalities within each person that IFS therapists classify into three broad categories: exiles (wounded parts), managers (preventive parts), and firefighters (reactive/extreme parts). Parts are separate from the Self.

Polarization: A conflict that typically, but not exclusively, occurs between two protective parts that often safeguard the same wound in an opposing manner. Polarizations can become extreme and costly, especially with trauma, and resolution occurs when the Self validates each side and parts allow the Self to resolve the dilemma.

Retrieval: One of the steps of the unburdening process. Once an exiled part is identified and given a corrective experience by the Self, it's free to leave the past and exist somewhere safe in the present day.

Self: An innate presence in each of us that promotes balance, harmony, and nonjudgmental qualities (such as curiosity, caring, creativity, courage, calm, connectedness, clarity, and compassion). The Self is a state of being that can neither be created nor destroyed.

Self-energy: The quality and essence of a client's Self that the therapist directs toward the client's internal parts and external relationships.

Unblending (separating): A mental state of differentiation where parts can be present for, yet remain separate from and not dominate or overwhelm, the Self.

Unburdening: The process of healing wounded parts or exiles. The steps include witnessing, do-over, retrieval, unload, invitation, and integration.

Unloading: One of the steps of the unburdening process. Once the wounded part is safely in the present, it is allowed to release the thoughts, feelings, and physical sensations it carried with regard to the trauma.

Updating: The process of introducing a part (protective or exiled) who is in connection with the Self of the past to the current-day Self.

Witnessing: One of the steps of the unburdening process. The exiled part can show and/or tell the Self about its experience in a way that allows the part to feel fully seen, understood, heard, and validated.

* Some of these definitions were taken from the *Internal Family Systems Skills Training Manual* (Anderson et al., 2017).

CHAPTER 2

Various Types of Trauma

Not all traumas are the same, nor should they be treated equally. The following section provides a brief description of the various types or categories of trauma. Typically, individuals can experience a single traumatic event or what is referred to as acute stress disorder and, for the most part, have a high likelihood of fully recovering from the experience within one month. However, if they have several traumatic events in their lives, they are more likely to experience chronic trauma and have symptoms of PTSD that persist beyond one month. When these traumatic experiences involve repeated relational violations in childhood, individuals can acquire developmental trauma disorder (DTD) or attachment trauma, and when these symptoms or violations persist or occur in adulthood, it can result in a diagnosis of complex PTSD or relational trauma. Under more extreme circumstances and following early, repeated, and/or severe abuse, a diagnosis of dissociative identity disorder (DID) is common, which consists of multiple discrete parts that operate independently of each other.

SINGLE-INCIDENT TRAUMA OR ACUTE STRESS DISORDER

Single-incident or acute trauma typically results from a lone traumatic event that occurs in someone's life, such as a car accident, sudden death of a loved one, physical assault, robbery, or acute medical illness. The resulting symptoms (e.g., intrusive images, negative mood, dissociation, avoidance, numbing, and arousal) are similar to those experienced by chronic PTSD. With acute stress disorder, however, the symptoms are normal physiological adaptations to an acute event that typically resolve within one month of the trauma occurring (SAMHSA, 2014). A survey conducted in 24 countries across the world found that over 70 percent of people report experiencing at least one traumatic event in their lifetime, and 30.5 percent report being exposed to four or more traumas (Benjet et al., 2016).

We should respond differently to acute trauma than to chronic trauma or complex PTSD. With acute trauma, it's more important to provide a safe holding environment so clients are naturally and organically able to process the event. PTSD symptoms are normal within the first month of a trauma and, to date, we don't have a specific protocol in IFS to treat acute trauma. However, I've seen IFS be extremely effective, especially when clients can access Self-energy. The Self provides the corrective nurturing environment for parts and helps them avoid taking on any extreme role of protection or the

need to carry the pain in isolation. *In acute trauma, the Self's presence can prevent the development of protective parts and exiled wounds.*

Eye movement desensitization and reprocessing (EMDR) is an established and well-known treatment for acute stress disorder and has sufficient research to support its use with acute trauma (Buydens, Wilensky, & Hensley, 2014). Critical incident stress debriefing (CISD) has historically been a common treatment for acute trauma, especially when it occurs in groups, but its efficacy has come into question, and the protocol is currently being reevaluated to determine the correct amount of processing that is beneficial after a traumatic event (Wessely & Deahl, 2003). Talking too much or not enough can both be problematic. Certain medications, such as benzodiazepines, should also be avoided during the acute phase of a trauma because they can interfere with normal memory processing and can increase the likelihood of developing chronic PTSD (Guina et al., 2015). There is, however, emerging research showing that steroid use can be helpful during the acute phase of trauma and might help prevent the development of chronic PTSD (Rasmusson et al., 2017).

PTSD OR CHRONIC TRAUMA

There are several known risk factors that determine whether or not someone will remain symptomatic and develop chronic PTSD after an acute traumatic event. Some include individuals who have experienced a prior trauma, those who have mental health-related issues, those who struggle with substance abuse, and those who have a genetic history of PTSD. Here is an overview of the diagnostic criteria for PTSD from SAMSHA's Treatment Improvement Protocol (TIP) Series (SAMSHA, 2014):

- a. Exposure to actual or threatened death, serious injury, or sexual violence
- b. Presence of intrusive symptoms associated with the traumatic event
- c. Persistent avoidance of stimuli associated with the traumatic event
- d. Negative alterations in cognition and mood associated with the event
- e. Marked alterations in arousal and reactivity associated with the event
- f. The duration of the disturbance is more than one month
- g. Symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning
- h. Symptoms are not attributed to the physiological effects of a substance or other medical condition

As we know, PTSD can result from a variety of different kinds of trauma, such as a car accident, the loss of a loved one, a date rape, witnessing someone else's trauma, or living through a global pandemic. It can result from a single acute event that turns chronic, or it can occur after several related events.

COMPLEX PTSD OR RELATIONAL TRAUMA

When trauma is repetitive and relational in nature, it's called complex PTSD or relational trauma. According to Judy Herman, the author of the classic book *Trauma and Recovery*, complex PTSD is the result of exposure to repeated or prolonged instances or multiple forms of interpersonal trauma, often occurring under circumstances where escape is not possible due to physical, psychological, maturational, family, environmental, or social constraints (Herman, 1992).

The International Society for Traumatic Stress Studies (ISTSS) developed a task force to define and develop treatment guidelines for complex PTSD. Their definition includes the core symptoms of PTSD in conjunction with disturbances in five domains of self-regulatory capacities, including: (1) emotion regulation difficulties, (2) disturbances in relational capacities, (3) alterations in attention and consciousness (e.g., dissociation), (4) adversely affected belief systems, and (5) somatic distress or disorganization (Cloitre et al., 2012). Bessel van der Kolk and others have alternately named the syndrome "disorder of extreme stress not otherwise specified (DESNOS)" (Pelcovitz et al., 1997).

This is the type of trauma that we will be primarily focusing on in this book. It is the most common form of trauma to show up in your office, and it's difficult to treat and likely to activate your parts. But when it's healed, it can have a dramatic effect on the quality of your clients' lives. I believe IFS is perfectly suited to this task because of its ability to provide relief and healing of wounded parts that carry the effects of repeated relational violations.

DEVELOPMENTAL TRAUMA

There are two different ways to view developmental trauma. One is more common and widely recognized. This is relational trauma or complex trauma in childhood, also known as developmental trauma disorder (van der Kolk, 2005). This perspective of developmental trauma is supported by the Adverse Childhood Experiences (ACE) study performed by Kaiser Permanente and the Centers for Disease Control and Prevention, which showed that 11 percent of respondents reported emotional abuse in childhood, 30 percent reported physical abuse, and 20 percent reported sexual abuse (Felitti et al., 1998). In addition, 24 percent reported exposure to family alcohol abuse, 19 percent were

exposed to mental illness, 13 percent witnessed their mother being battered, and 5 percent reported family drug abuse. These numbers are alarmingly high and much more common than we previously acknowledged.

Felitti and colleagues also found a high correlation between a person's ACE score and negative health outcomes later in life, such as depression, suicide attempts, alcoholism, drug abuse, sexual promiscuity, domestic violence, cigarette smoking, obesity, physical inactivity, and sexually transmitted diseases. Additionally, individuals with higher ACE scores had a higher risk of heart disease, cancer, diabetes, skeletal fractures, and liver disease. This alarming report is consistent with the view of Gabor Maté, author of *When the Body Says No: Exploring the Stress-Disease Connection*, who maintains that most medical and mental health problems are rooted in trauma.

Another way to approach developmental trauma is to ask the question: *At what developmental stage did the trauma occur?* This perspective looks at the impact a traumatic experience has on brain development, symptom production, and psychiatric diagnosis. Martin Teicher, the director of the Developmental Biopsychiatry Research Program at Harvard Medical School, and his lab have shown that childhood maltreatment alters brain structure and function, depending on the type of abuse and stage of development. His group identified “sensitive periods” when certain brain regions are maximally vulnerable to different types of trauma. For example, they have demonstrated how verbal abuse is associated with alterations in the auditory cortex and language production and how witnessing domestic violence is connected with changes in the visual cortex and the limbic system. Their research shows us how various traumas, at different ages, affect different parts of the brain and create different symptoms and different psychiatric disorders (Teicher & Samson, 2016)

Another pioneer in the study of developmental trauma is Bruce Perry, the creator of the Neurosequential Model of Therapeutics. He looks at the ways trauma and neglect change the biology of the brain and affect childhood cognitive, behavioral, emotional, social, and physiological development. As we continue to identify which brain regions are affected by different types of trauma at various developmental stages, we can begin to develop brain-specific interventions to help treat and potentially reverse the physiologic abnormalities created by each specific trauma. It's important to remember that we never stop developing. A traumatic loss will affect us differently at age 12 versus age 30 or 60 because of its differing effect on our brain, body, and emotional capacity at each developmental stage.

EXTREME OR DISSOCIATIVE TRAUMA

The last type of trauma is extreme or dissociative. I personally see dissociation as a *spectrum disorder*, meaning that there is often a range of dissociative parts, from the most benign (e.g., a part spacing out) to the most extreme (e.g., many different parts appearing separate from one another with no overlapping memory or time awareness, as is the case in DID). With trauma, dissociative parts are often extreme in nature, as they respond to what they believe is a life-or-death situation and then correspondingly disengage, detach, or numb out as a survival mechanism. (More on this later as we explore the neurobiology of PTSD and dissociation.)

Many therapists get easily overwhelmed by clients who struggle with DID because they can have parts in their system that appear disruptive, are confusing, and are often extreme in presentation. They sometimes present with one unsolvable dilemma after another but know this is all in the service of protection. It's important to stay steady with the IFS principles and not get too caught up in the drama; some parts protect, some parts carry wounds, and, somewhere deep inside, Self-energy helps heal the pain.

The following chart provides a summary of the different categories of trauma.

Acute Stress Disorder	• Resolves within the first month. • Reexperiencing, avoidance/numbing, and hyperarousal are normal adaptations. • Benzodiazepines can increase likelihood of chronic PTSD.
PTSD	• Symptoms persist beyond one month. • Intrusive images, avoidance, negative beliefs, increased arousal, and reactivity. • First trauma primes system for chronic PTSD.
Complex PTSD or Relational Trauma	• Repeated interpersonal violation with no escape. • Core PTSD symptoms plus difficulties with emotion regulation, relational difficulties, alterations in attention and consciousness, negative beliefs, and somatic distress. • Also called DESNOS.
Developmental Trauma	• Relational trauma in childhood or developmental trauma disorder (DTD). • At what developmental stage did the trauma occur? • Brain region affected correlates with symptom production.
Extreme or Dissociative Trauma	• Dissociative identity disorder (DID) • Life threat and escape • Dissociation along a spectrum

CHAPTER 3

IFS: Permanent Healing of Emotional Wounds

THE MODEL

I believe that the IFS model is unique in some of the ways it supports healing, especially as it relates to trauma. I also find it the most comprehensive and the best fit for my personality and style of psychotherapy. Here is a brief overview of the IFS model, in case you are new to IFS. For a complete overview, including step-by-step explanations, exercises, and clinical examples, please refer to the *IFS Skills Training Manual* (Anderson et al., 2017).

I have a great deal of admiration for IFS founder Dick Schwartz and the way he allowed (and continues to allow) the model to unfold organically. He is steadfast in staying true to what his clients have told him about their internal experiences. Dick enabled IFS to evolve in a grassroots fashion, serving as a model of the people.

THREE MAIN PARTS

In IFS, there are three types of parts we commonly encounter: (1) exiles or *parts that carry wounds*, (2) managers or *preventive parts*, and (3) firefighters or *extreme, reactive parts* (Schwartz & Sweezy, 2020). Each of these parts is a normal aspect or component of our client's personality until it needs to take on a specific role within the system (i.e., to protect or to carry a wound as a result of some overwhelming or traumatic life experience). In this book, we will refer to the manager protective parts as *preventive* and the firefighter protective parts as *reactive* or *extreme*, and we will refer to the exiled parts as *wounded* or *those who carry the pain*.

I often refer to *preventive parts* as the unsung heroes. They run our day-to-day life, they're hard-working and often exhausted, and they don't get the credit they deserve. They rarely get a break, and these are the parts that love taking medications in search of some relief. Some common examples are pleasing parts, parts that want to be liked, and parts that are funny, obsessive, or conflict-avoidant or

that want to look good. They tend to have a hard time separating or unblending because they run the show and often will say things like: “What do you mean by ‘separate’? I am Lily.” They like to feel and appear as if they’re in charge and under control.

Extreme or reactive parts are plentiful in clients who have suffered from trauma and tend to be the most challenging and troublesome for therapists. Remember: Extreme circumstances require extreme responses. In treating relational trauma, which tends to be long-term, repetitive, and ongoing, these parts are often tenacious, absolute, and highly triggering to those who encounter them. Most of the psychotherapy consultations I provide for therapists are about their own parts that get activated by extreme parts of clients.

Society, culture, partners, and family members tend to hate these extreme, reactive parts (and these parts know it too). But continuing education organizations and conference organizers tend to love them because therapists are constantly looking for new ways to work with these challenging parts. Some examples are cutting parts, suicidal parts, parts that numb out and dissociate, parts that abuse substances, parts that overeat or under eat, parts that shame, and parts that are critical or perpetrate. The list can go on and on.

These diverse and numerous parts are actually among my principal motivations for writing this book. I have a deep desire to assist therapists in guiding their clients to overcome relational trauma, but in order to help someone heal, we need to be calm, efficient, composed, and confident when encountering these extreme parts in order to get their permission to access the wound.

The *wounded parts* are the young, sensitive, and vulnerable aspects of our personalities. They were forced to carry the hurt, pain, sense of betrayal, sadness, loneliness, shame, neglect, and lack of love as it related to the painful and difficult moments in our lives. We can, of course, get hurt and sustain wounds at any stage of our lives. Relational trauma can occur in childhood from abusive caregivers or bullies at school, as well as in adulthood from abusive partners or unhealthy colleagues at work.

Painful experiences can be held and expressed in many different forms. Parts can communicate through body sensations, emotions, thoughts, or beliefs. They can reveal what they are holding through visual, auditory, or other sensory modalities. It’s important for therapists to help clients align with the part’s preferred method of communication. For example, they can *listen* to a part that communicates through sound or *see* a part that expresses itself visually. If a client tries to *hear* what a more tactile part is expressing through *physical sensation*, communication is misaligned, and the desired result will not be achieved.

I believe that exiles live primarily in implicit memory, and flashbacks are wounded parts sharing fragments of what they're holding through one of the five senses. After all, wounds do not define parts; when we help them unload what they carry, they once again become the loving, fun, and creative aspects of our personality that were always there but were temporarily obscured by the trauma.

WE ALL HAVE A SELF

One of the great contributions of the IFS model to the field of psychotherapy is the recognition of the Self. Dick Schwartz didn't invent Self-energy, but he certainly brought awareness to it by attentively listening to his clients with an open heart and genuine interest.

We have further clarified the Self as being different from parts. As mentioned earlier, we are each born with a Self that possesses an internal wisdom and the natural intrinsic capacity to heal. It has been called the core seat of consciousness—our soul, a spiritual space that is connected to love, nature, and the divine. The IFS model describes the 8 C's of Self-energy as: (1) calm, (2) curiosity, (3) compassion, (4) connectedness, (5) creativity, (6) courage, (7) clarity, and (8) confidence (Schwartz & Sweezy, 2020). I describe the Self as *a state of being*; one truly “gets it” when they experience Self-energy or embody it personally. It can be experienced differently by different people. For me, it was initially a theoretical construct until halfway through my Level 2 training. I remember the moment when I felt the Self for the first time. It was like a tingling energy and simultaneous sense of calm moving throughout my whole body.

DIFFERENT FROM PHASE-ORIENTED TREATMENT

Most psychotherapy modalities that focus on treating trauma survivors support a phase-oriented approach. The ISTSS Expert Consensus Guidelines for Complex PTSD describe three distinct, well-recognized phases:

- Phase 1 focuses on enhancing the individual's safety, reducing symptoms, and increasing important emotional, social, and psychological competencies.
- Phase 2 focuses on processing the unresolved aspects of the individual's memories of the traumatic experiences. This phase emphasizes the review and reappraisal of traumatic memories in order to integrate them into an adaptive representation of one's self, one's relationships, and the world.

- Phase 3, the final phase of treatment, involves the consolidation of treatment gains to facilitate the transition from the end of the treatment to greater engagement in relationships, work, education, and community life (Cloitre et al., 2012).

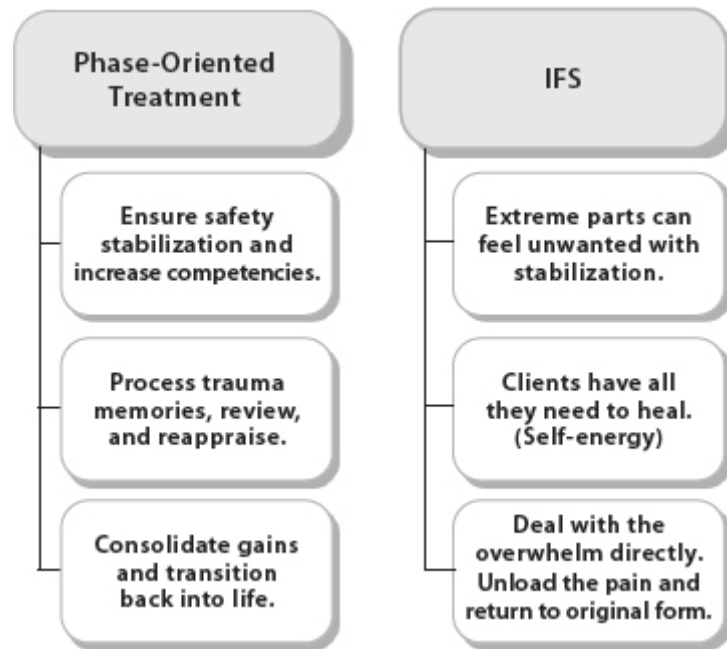
The IFS model endorses certain steps that help achieve the desired outcome in a slightly different way. For example, we follow 6 F's when working with protective parts: (1) find, (2) focus, (3) flesh out, (4) feel toward, (5) befriend, and (6) find out the fear. We also abide by the six steps of the unburdening process: (1) witness, (2) do-over, (3) retrieve, (4) unload, (5) invitation, and (6) integration (Schwartz & Sweezy, 2020). However, IFS doesn't subscribe to a phase-oriented treatment approach for several important reasons. First, the message can be off-putting to some protective parts. For example, phase one (safety stabilization) embraces certain qualities of some parts more than others. A therapist might say: "I think it would be a good idea if you would meditate more, try eating healthier, and start exercising again." The suicidal, depressed, and binging parts of the client hear this as: "She's not interested in us or what we have to say or how we are trying our best to help. She cares more about those parts than she does about us." *Phase one can give the wrong message to hard-working extreme parts: that they're really not wanted.*

Another difference between the IFS view and the phase one suggestion of increasing competencies is that, in IFS, we believe everyone has a Self; we are born with what we need (Schwartz & Sweezy, 2020). Self-energy has innate healing capacities that unfold naturally. Many clients believe they have an internal deficit, exclaiming: "I'm broken. I'm empty. I have no Self." In these instances, I politely reply: "We can agree to disagree. I believe you have all that you need inside to heal your trauma. I am here to help you *access it*, not cultivate or grow it."

Another important difference between IFS and the phase-oriented approach is that IFS offers a way to deal directly with overwhelming feelings (see [chapter 9](#)), as opposed to building up strengths and resources the client can use to manage the intensity of the traumatic effect. Dick Schwartz taught us that if you talk directly to the part that holds the overwhelming feelings and ask it not to overwhelm the client, it will comply in order to receive help and experience relief.

Last, IFS teaches that healing is connected to helping burdened parts release or unload traumatic feelings and experiences they've been forced to carry, allowing them to return to their pre-burdened existence. This is different from the phase-oriented treatment method of processing, reviewing, and reappraising traumatic memories to integrate the experience into an adaptive representation of the self. In a conversation I had with him, Dick described IFS as a constraint-release model of healing as opposed to a review-reappraisal model of healing.

I remember my first exposure to IFS, which was during a one-day introductory workshop that Dick gave at one of Bessel van der Kolk's annual trauma conferences. I had been working in the trauma field for several years at this point and was used to the concept of parts. Our treatment goal was all about helping our clients process their traumatic experiences and having their parts work together in a harmonious way—consistent with the phase-oriented perspective. When I heard Dick talk about healing the wound and getting the part out of the past and into the present, it was a game changer for me. This was a totally new idea for our field, and I was hooked and wanted to learn more.



CHAPTER 4

Integrate IFS into Your Treatment Approach

DIFFERENT WAYS TO TREAT TRAUMA

Several different psychotherapy modalities can be effective for the treatment of complex PTSD, each with their unique dimensions, including IFS, EMDR, sensorimotor psychotherapy (SP), somatic experiencing (SE), and accelerated experiential dynamic processing (AEDP). Most of these models go beyond the cognitive domain and are referred to as experiential therapies, meaning that the body, emotions, and relationships are central, and clients have the resources to heal.

Many therapists who want to learn how to treat complex PTSD effectively will train in several of these modalities. Most of us end up with our own approach, which is usually an amalgam of different aspects of various models that we uniquely combine to create a way of successfully working with clients.

INTEGRATING IFS: WE ALL HAVE OUR OWN MODEL

The way I teach how to integrate IFS with other models is to proceed with an overall IFS framework as you incorporate different aspects of other models. For example, assume that everyone has parts, including parts that protect and parts that carry wounds, and that we each have inherent wisdom and healing capacity to mend our scars: the Self. However, don't proceed with any intervention without getting permission from the client's parts first. It's okay to apply different methods for treating clients and to utilize the clinical knowledge you've accumulated over the years, but it's important to incorporate them carefully in treatment. This requires letting go of your agenda and interpretations and trusting the client's Self-wisdom. *Remember: Never proceed without receiving all parts' permission first.*

For example, you may want to set up an EMDR protocol with a particularly stubborn protective part that is not willing to separate, to do somatic work with a part that looks like it is carrying a lot of physical activation in the body, or to apply cognitive behavioral therapy (CBT) with a part that is holding on to a distorted belief. You may also want to engage in AEDP, a type of relational therapy,

with a young wounded part that seems desperate for connection. But always remember to ask the client to check inside and see if their parts are willing to separate, and make sure all of their parts agree before proceeding.

In my own experience, I trained first in EMDR and next in SP. After I learned about IFS, I started noticing that any EMDR protocol I established was usually with a protective part that was unwilling to separate. When identifying negative cognitions in EMDR, I was careful to first focus on protector-related beliefs (such as “I’m angry” or “I’m an alcoholic”) instead of exiled beliefs (like “I’m unlovable” or “I’m worthless”). I found that doing an EMDR protocol with wounded parts without permission from protective parts resulted in unwanted abreactions or protector backlash.

The same was true regarding SP. I would embark on a piece of body work with a client and begin to notice that I was again dealing with a particular protective part. I carefully observed how the part held posture in my client’s body, the way it expressed itself through physical sensations, and the movements it needed to release what it was holding. Early in my IFS career, I would often turn to EMDR and SP when I was stuck or unable to help my client move forward with IFS. Fortunately, the more proficient I became with IFS, the less I relied on the other modalities to help my clients salve their emotional scars.

WHAT’S IMPORTANT FOR HEALING?

Putting each model aside for a moment, I’ve spent a good deal of time thinking about the key elements or necessary components for healing trauma. Why are some clients able to accomplish this goal with relative ease while others take decades to let go of their burdens? How much of this is about their inherent capacity versus the severity of the trauma they’ve endured? What are the essential elements required for permanent healing to occur?

One thing I’ve consistently observed in most, if not all, effective trauma treatments is separation. In other words, being with, not within, one’s experience appears to be a key component to healing. I often say and clinically observe that *nothing therapeutic happens in reenactment*. Neuroplasticity holds that every time a network fires, it strengthens and grows. Therefore, reliving or reenacting the dysregulated neural networks of trauma reinforces them. Mindful separation, on the other hand, can support the necessary distance, perspective, and environment for healing to occur. IFS, EMDR, SE, and SP all have a “be with” component built into their models. For example, EMDR incorporates the phrase “sit on the bus and watch the trauma go by.” SP instructs clients to drop the content of the story and notice their physical sensations. IFS encourages parts to step back, relax, and allow separation from the Self.

We also know that the therapeutic relationship and quality of the connection between the therapist and the client are extremely important. Creating the necessary level of comfort allows wounded parts to safely expose their vulnerability in the service of recovering from the trauma.